



HEALTH AND SOCIAL CARE SCRUTINY COMMISSION

MINUTES of the Health and Social Care Scrutiny Commission held on Monday 1 December 2025 at 7.00 pm at 160, Tooley Street, SE1 2QH

PRESENT: Councillor Suzanne Abachor (Chair)
Councillor Maria Linforth-Hall (Vice-Chair)
Councillor Esme Dobson
Councillor Sandra Rhule
Councillor Charlie Smith
Councillor Naima Ali

**OTHER MEMBERS
PRESENT:**

**OFFICER
SUPPORT:** Pauline O'Hare, Director of Adult Social Care

David Bass, Highways Technician

David Sole, Parking Services and Development Manager

John Wade, Head of Parking Services and Network Management

Michelle Peake, Head of Specialist Services

Julie Timbrell, Project Manager , scrutiny

1. APOLOGIES

Councillor Charlie Smith gave apologies for lateness.

2. NOTIFICATION OF ANY ITEMS OF BUSINESS WHICH THE CHAIR DEEMS URGENT

There were none.

3. DISCLOSURE OF INTERESTS AND DISPENSATIONS

There were none.

4. MINUTES

The minutes of the meeting held on 16 October were agreed as an accurate record.

5. HOARDING REPORT

The **Chair** introduced the item, advising that a presentation on hoarding in Southwark had been provided as evidence to support the Health and Social Care Scrutiny Commission's review of safeguarding.

The Chair invited **Pauline O'Hare , Director of Adult Social Care,** to introduce herself and present the report.

Presentation Summary

The Director of Adult Social Care delivered a presentation on hoarding in Southwark, setting out the nature of hoarding behaviour, its impact on individuals and communities, and the Council's multi-agency approach to prevention, risk management and safeguarding.

Members were informed that hoarding disorder is characterised by a persistent difficulty discarding possessions, regardless of their value, and is often accompanied by significant distress. Hoarding may exist as a condition in its own right or alongside other physical

or mental health conditions, including dementia, depression, substance misuse, schizophrenia, bipolar disorder, learning disability and obsessive-compulsive disorder. It was emphasised that hoarding can affect individuals of all ages and backgrounds and does not always indicate a lack of mental capacity.

The presentation outlined different forms of hoarding, including inanimate object hoarding, animal hoarding, data hoarding and financial hoarding. Officers described common characteristics, such as anxiety, excessive emotional attachment to possessions, difficulty making decisions, social isolation and limited insight into risk. Reference was also made to Diogenes Syndrome, a severe form of domestic squalor associated with extreme self-neglect, which may be primary in nature or secondary to other mental health conditions.

The legal framework for intervention was explained. Members were advised that hoarding is recognised under the **Care Act 2014** as a form of self-neglect and may therefore give rise to safeguarding duties where an individual has care and support needs and is unable to protect themselves from harm. The **Mental Capacity Act 2005** was highlighted as relevant when assessing an individual's ability to understand and weigh the risks associated with hoarding behaviour.

Members were informed about the **Southwark Hoarding Panel**, which meets every six weeks and brings together Adult Social Care, Housing, Children's Services, South London and Maudsley NHS Foundation Trust (SLaM), the London Fire Brigade and other partners. The panel reviews live cases and supports officers through a long-term, trauma-informed and multi-agency approach, recognising that sustainable change often requires ongoing engagement.

Data presented suggested that between 3,000 and 6,000 residents in Southwark may be affected by hoarding, although only a small proportion are likely to seek specialist support. Panel data showed a gradual increase in known cases among council tenants over recent years. Officers emphasised that without assessment and sustained

intervention, long-term improvement is unlikely.

The presentation also outlined the roles of key partners. Adult Social Care can provide support where Care Act eligibility is met; Environmental Health and Housing can intervene where conditions pose risks to health or safety; the London Fire Brigade carries out home fire safety visits; and health services, including SLAM, have responsibilities to assess and treat hoarding as a recognised mental health condition. The importance of training, trauma-informed practice and coordinated working across agencies was emphasised throughout.

Questions and Discussion

Members raised a number of questions following the presentation.

- **Ongoing support and mental health treatment**
A member asked whether individuals who hoard and have mental health conditions continue to receive support once treatment has ended. Officers advised that this depends on the severity of the hoarding behaviour and the individual's willingness to engage. Once immediate risks have been addressed, if an individual does not wish to continue to engage with services, ongoing support may not always be possible.
- **Balancing individual rights and wider impacts**
Members commented on the tension between respecting an individual's rights and protecting the safety and wellbeing of family members and neighbours. Officers explained that where a person has eligible needs under the Care Act, support such as deep or "blitz" cleans may be offered to enable care delivery or to maintain safe access within a property. However, it was stressed that blitz cleans can be traumatising and may worsen hoarding behaviour, and that trauma-informed practice is therefore prioritised.

- **Consistency between policy and practice**

A Councillor raised concerns that, in her experience, cases were not always handled in line with the practices described. She referred to a constituent living in extreme clutter and unsanitary conditions that had been described as a “lifestyle choice” by an Occupational Therapist. Officers agreed that hoarding should not be characterised as a lifestyle choice and stated that it is more commonly linked to trauma experienced in childhood or adolescence. The Member said in this case it had taken approximately two years to secure care support. Officers noted that while they could not comment on individual cases, engagement can take a significant amount of time and often requires a trigger for intervention, such as vermin, a fall or other serious risk. Members asked whether training was mandatory; officers advised that training was not mandatory but that extensive training opportunities were available.

- **Housing enforcement and eviction risk**

Members raised concerns that threats of eviction can exacerbate hoarding behaviour. Officers responded that Housing services sometimes need to act where there are serious risks, such as fire hazards or infestations, and that intervention may be necessary for the greater good. It was suggested that a combination of incentives and enforcement can sometimes be effective. A Councillor described a further case in which the housing officer appeared insufficiently informed about addiction issues and suggested that additional training was needed. Officers agreed that further training would be beneficial.

- **Hoarding in social housing**

Members asked how hoarding is addressed within social housing. Officers explained that there may be less need for intervention in certain situations, for example in semi-detached properties, but that referrals are frequently made by family members or neighbours. Where concerns relate only to clutter or mess, or a lot of possessions, officers may advocate for an individual’s right to hoard. However,

where conditions become unsanitary or impact neighbouring properties, intervention would be required.

- **Referrals and specialist support**

Officers confirmed that referrals are received from Housing Associations and that support may include blitz cleans and the involvement of specialist external agencies. Engagement was described as highly sensitive, requiring careful exploration of an individual's experiences and emotions, similar specialist support provided in the TV programme *The Hoarder Next Door*. Officers noted that cases involving unsanitary conditions are often the most challenging.

- **Early intervention and outcomes**

A member described a case involving a hoarder with care needs who eventually received effective support and subsequently experienced improved stability and reduced hospital admissions. The Member noted that health professionals had visited the property for a prolonged period before action was taken and suggested that intervention could have occurred earlier. She also highlighted that successful intervention had generated cost savings for health and care services. Officers acknowledged the importance of earlier intervention where possible.

Conclusion

The Commission noted the presentation and the discussion. Members highlighted the importance of consistent application of guidance, trauma-informed practice, earlier intervention, and appropriate training for frontline staff, and recognised hoarding as a complex safeguarding issue requiring sustained, multi-agency working.

6. BLUE BADGE SCHEME

The following officers introduced themselves and contributed to the discussion:

- **David Bass**, Highways Technician
- **David Sole**, Parking Services and Development Manager
- **John Wade**, Head of Parking Services and Network Management
- **Michelle Peake**, Head of Specialist Services

The chair invited the officers to provide a verbal summary of the report:

Officers summarised the actions taken in response to the Commission's earlier recommendations, including improvements to website information, an overview of Blue Badge fraud activity and enforcement, clarification of the differences between Blue Badges , Disabled Resident's Permit, AD permits and Streetscape permits and an outline of the current process and timescales for providing these. Officers also highlighted ongoing work to review systems and processes in order to improve integration and the resident experience.

The chair invited questions and the following points were made:

Linking AD Permits, Streetscape , Disabled Resident's Permits

A member asked whether it would be possible to link the disabled resident permit application process so that an AD permit and Streetscape permits could be issued automatically. Officers explained that work was underway to explore better integration of systems but noted that different permits operate under separate frameworks and system constraints, which would need to be addressed.

Communication and clarity between AD permits and Blue Badges

A member highlighted confusion arising from the similar appearance of renewal emails relating to AD permits and Blue Badges, which can cause misunderstanding for residents. Officers undertook to review this issue.

Display notices for legitimate Blue Badge use

A member suggested reintroducing a notice for display in vehicles to show legitimate ownership of a Blue Badge, noting that this had previously been provided before being discontinued during the Covid-19 pandemic. Officers confirmed that such notices had been provided historically but advised that there would be a cost implication. Officers also noted that there had been no recent requests for this feature and that any reintroduction would need to be weighed against efficiency and value for money considerations. Members commented that Blue Badges had become increasingly valuable, which is why the AD permit system had been introduced

i.e. in order to reduce the theft of Blue Badges. However, the lack of any notice on display in a vehicle means that other road users cannot recognise that the driver is a Blue Badge holder. This results in other road users thinking that a Blue Badge holder is eg illegally parking on double yellow lines when they are not, and other Blue Badges holders mistakenly thinking that a non-disabled driver is parked in a disabled bay when in fact they are a Blue Badge holder. The reintroduction of the paper scheme could eliminate such tensions between road users.

London-wide approach to AD permits

A member asked whether it would be possible to introduce a London-wide scheme for AD permits. Officers agreed to raise the issue through London Councils, while noting that approaches differ across boroughs. It was explained that Southwark operates the national scheme, whereas other boroughs offer additional local allowances.

Deterrence of Blue Badge fraud

In response to a question about deterring fraud, officers commented that ongoing enforcement action acts as a deterrent. Michelle Peake added that a public awareness campaign was being considered. Officers advised that the most common causes of fraud were stolen badges and, less frequently, use of badges belonging to deceased individuals.

Motability vehicles and enforcement correspondence

A member raised concerns about cases involving Motability vehicles, where traffic enforcement notices are issued to the disabled person as the registered keeper, and difficulties arise when carers attempt to resolve matters on the resident's behalf. Officers stated that, in such cases, they would be willing to discuss matters with an appropriate third party. The member described a long-running case in which increasingly threatening letters had been sent to a young man who is non-verbal, cannot write and cannot drive but he was the registered owner of the vehicle. She noted that a key issue was that membership of the mobility scheme is not always apparent, and officers agree this was a difficulty. The member asked for clarification as to why the parking department needed the resident to prove that they were disabled when ownership of a Motability vehicle de facto means that the resident is disabled. Officers explained that they do not have access to this information on their records.

Training for officers was suggested, and officers agreed that additional training would be beneficial.

RESOLVED

Officers will be asked to provide follow up on :

- An update on improving communication to residents to avoid confusion regarding the Blue Badge and AD Permit application process.
- Discussions with London Councils on developing an integrated AD Permit scheme
- A consideration of the request to reintroduce a display notice to indicate to the public that an AD Permit means the resident has legitimate right to disabled parking and Blue Badge
- An update on training to resolve potential communication problems between Motability residents / carers and the Council

7. SCRUTINY REVIEW : CANCER PREVENTION AND EARLY DIAGNOSIS

The commission discussed the headline report, and the following points were made:

- Prostate cancer rates are high, particularly amongst the black community. There is presently more publicity about this but this could be magnified even more. There has been a recent debate about a national screening programme but this has not been agreed, however there is definitely a case for more awareness and education around the non-invasive blood tests that can be carried out.
- A member spoke about the difficulties of getting an early diagnosis for blood cancer and spoke about a case where various opportunities were missed.

RESOLVED

- More information will be requested on blood cancer early diagnosis rates and more evidence on good practice from specialist charities sourced.
- The recommendation on the proposed self-sampling kits and language-specific information packs for women with FGM will be removed as members did not think this would work as the

trauma of FGM often causes physical and psychological scars, including bodily alienation, which would make self-sampling difficult.

- The recommendation on more work to prevent cancer will be strengthened.

8. HEALTHWATCH SOUTHWARK

The Healthwatch Southwark's Annual Report 2024-25, alongside a report setting out priorities for 2025-26, was noted.

Members asked about the future of Southwark Healthwatch given recent government announcements .

The scrutiny project manager conveyed an update from Healthwatch Southwark, who advised that they are:

- asking for support in signing this national petition: <https://petition.parliament.uk/petitions/732993>
- asking for support in terms of people reaching out directly to their MPs, to reflect the need for our services.
- provided a webpage of information about what Southwark Healthwatch understand the changes to mean: <https://www.healthwatchsouthwark.org/future-healthwatch-southwark-what-you-need-know>
- are waiting for Department of Health and Social Care confirmation of future funding post this financial year, update expected in January or after the King's speech
- are trying to influence the DHSC's new Directorate of patient experience as a collaborative network: <https://www.kingsfund.org.uk/insight-and-analysis/projects/learning-from-healthwatch>

9. WORK PROGRAMME

The workplan was agreed with the addition of :

- Interview with the Cabinet Member for Health and Well-being
- GP Appointments – report back on engagement with local practices to improve access to timely appointments

